BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

Check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Blood / kidney / liver	full blood count (FBC), renal and liver profile	Confirm treatment-day results	Repeat on the source/local schedule	Review unresolved abnormalities	Individual clinical plan
Heart / breathing	Baseline cardiac assessment	Reassess before trastuzumab	Echo/multigated acquisition (heart pumping) scan (MUGA) every 12 weeks	Keep clock through phase change	Product-information / risk-based plan
Cancer review	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Breast follow-up
Other checks	Confirm HER2 and baseline eligibility	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00316 v6 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · EMA Herceptin product information

THE WHOLE TREATMENT JOURNEY

Dose-dense AC → fortnightly paclitaxel + weekly trastuzumab: treatment and checks

HER2-positive neoadjuvant or adjuvant high-risk breast cancer

Four AC cycles every 14 days, then four fortnightly paclitaxel treatments with trastuzumab weekly on Days 1 and 8 of each 14-day block (eight weekly antibody doses). Trastuzumab then continues every 21 days to complete one year total; it is not given with AC.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer medicines NCCP 00316	Confirm exact regimen	Dose-dense AC → fortnightly paclitaxel + weekly trastuzumab	Four AC cycles every 14 days, then four fortnightly paclitaxel treatments with trastuzumab weekly on Days 1 and 8 of each 14-day block (eight weekly antibody doses). Trastuzumab then continues every 21 days to complete one year total; it is not given with AC.	Review active medicines and purpose	Next treatment / follow-up plan
Surgery / radiotherapy NCCP 00316	HER2-positive neoadjuvant or adjuvant high-risk breast cancer	Follow the multidisciplinary team (MDT) sequence	Radiotherapy timing needs agreement	Selected local treatment as indicated	Review recovery and ongoing effects
Blood / kidney / liver NCCP 00316	FBC, renal and liver profile	Confirm treatment-day results	Repeat on the source/local schedule	Review unresolved abnormalities	Individual clinical plan
Heart / breathing NCCP 00316 · SMPC	Baseline cardiac assessment	Reassess before trastuzumab	Echo/MUGA every 12 weeks	Keep clock through phase change	Product-information / risk-based plan
Cancer review NCCP 00316 · EARLY	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Breast follow-up
Other checks NCCP 00316 · BONE	Confirm HER2 and baseline eligibility	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

What each check is for

1 Baseline review

Before treatment

HER2-positive neoadjuvant or adjuvant high-risk breast cancer. Review lifetime anthracycline exposure and baseline cardiac risk before AC; reassess before starting trastuzumab. Assess cardiac function before trastuzumab and every 12 weeks during treatment. Keep the same monitoring clock when paclitaxel ends or trastuzumab changes schedule; arrange earlier review for symptoms or falling function. EMA product information recommends cardiac checks every 6 months after treatment until 24 months; following anthracyclines in early disease, yearly up to 5 years from the last trastuzumab dose or longer if function falls. Your team individualises this plan.

Ask / **act:** Confirm source eligibility and results.

Sources: NCCP 00316 · SMPC

2 Treatment-day check

Before each planned dose

Baseline blood tests and review before each chemotherapy administration according to the source and local plan. Once chemotherapy ends, testing follows trastuzumab and individual clinical needs.

Ask / **act:** Confirm active medicines and whether a dose is held.

Sources: NCCP 00316

3 Support plan

At every phase or day change

Four AC cycles every 14 days, then four fortnightly paclitaxel treatments with trastuzumab weekly on Days 1 and 8 of each 14-day block (eight weekly antibody doses). Trastuzumab then continues every 21 days to complete one year total; it is not given with AC.

Ask / **act:** Choose the matching support script; do not merge phases.

Sources: NCCP 00316

4 Response and follow-up

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to look for distant recurrence in a person without symptoms or clinical concerns.

Ask / **act:** Review symptoms, results and patient priorities together.

Sources: NCCP 00316 · EARLY

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.18